Nursing Study Platform — Merged Strategy Proposal

Oct 30, 2026 · @christian

Executive decision

Build Cueory (working name) as a free, faceless, beautifully designed nursing study brand over the next 30 days, spending no more than $100, and let donations and merch decide when paid tiers and the iOS app arrive. Launch Monday, November 2 with the "How to study for nursing school" mini-course and the brand; ship the first med-surg lesson the following week once it has passed the review workflow. Decide on December 20 using the gate numbers in this document.

This is version 2 of the proposal. It merges the Argon 4 and GPT 6 Pro strategy drafts, the GPT 6 Pro branding review and content-review policy, and Christian's 29 directions of October 30. The biggest changes from version 1: no founder on camera, free until a full course exists, a $100 cap instead of $1,000, a tiered review workflow instead of an RN on every item, autonomous AI agents for the back end, three shorts a day, a 3D body as the signature visual, and a study-skills course as the first product.

The product in one sentence: nursing study, made clear and already made, with a signature 3D body, run by an honest brand that tells you exactly what was checked and by whom.

Where the two proposals disagree, and which side this plan takes

The Argon 4 proposal is the more ambitious; the GPT 6 Pro proposal is the more careful. This plan takes the careful side on scale, legal boundaries and review, and the ambitious side on design, automation and the visual identity. Where Christian's own directions settle a question, they win.

Question	Argon 4	GPT 6 Pro	This plan
Question bank size	50,000 questions, cloud CAT	180 reviewed items in month one	180. Every item needs a human pass; 50,000 at 5 minutes each is about 4,170 hours

Question	Argon 4	GPT 6 Pro	This plan
Adaptive engine	Item Response Theory CAT from day one	Rule-based adaptive practice, labeled beta	Rule-based beta; calibrated testing later with real response data
Clinical review	Not addressed	Version 1: an RN on every item. Policy doc: tiered risk, AI checks plus human editorial pass, professional review only for higher-risk items	The policy doc's tiered workflow, with honest labels
Pricing	$1 / $3 / $5 tiers plus display ads	Free account, then a $5 Finals Pass	Free until a full course exists; donations and merch now; tiers after
AI operation	Six autonomous agents on a self-hosted server	Six role prompts, one human publisher	Autonomous agents for build, checks and content drafts; Christian approves what goes public
Founder on camera	Not required	Occasional founder updates	No face at all; a 3D body and a signature motif carry the brand
Video voice	Self-hosted TTS made to sound lifelike	One licensed voice, original visuals	One consistent AI voice, AI label on every video
Hosting	Oracle Cloud free tier, self-managed	Cloudflare Pages, Supabase, hosted checkout	GPT's managed stack, inside free tiers
Brand name	Argon, Vena, Soma, Aura Clinical, Lumen RN	Cueory, LessonForm, Cluerie	Cueory, Christian's pick, with a clearance search and Cluerie as backup

Question	Argon 4	GPT 6 Pro	This plan
Monday, Nov 2	Full vertical slice in 72 hours	One reviewed fluid-balance lesson or a labeled preview	Study-skills mini-course and brand launch; med-surg lesson the next week
Long-term	Skills kits, sim labs, accredited school, med school	Same stages, each gated on proof	Same stages, gated; brand partnerships (Figs, Alo, Lululemon) in the same gated list

What all three sources agree on and this plan keeps: the finals-season pivot, atomic sentences as the teaching unit, the multi-mode study engine, Open RN as the legal content base, the soft account prompt instead of a login wall, honest labeling of what was reviewed, and the calm editorial look.

Audience, timing and positioning

The first campaign is Med-Surg Finals: Core Concepts, aimed at students in their first year of a nursing program and at later-semester students who need the same foundations for exit exams. The pivot from TEAS is a judgment about timing and about Christian's own experience, not evidence that med-surg is the biggest market. TEAS and HESI A2 prep stays in the plan as the January release, when entrance-exam season picks up.

The window is real but narrower than a universal countdown suggests. Maricopa Community Colleges' general final-exam week is December 14–18, 2026; Central Arizona College's 16-week term ends with finals November 30–December 5. Individual nursing courses set their own dates. So the site asks each student for their exam date and builds the review schedule around it, instead of showing one six-week clock to everyone.

Positioning: med-surg, made manageable. Short explanations, practice that is already made, and a clear next step before finals. The shareable promise is "I finally understood it, and the practice was already made."

What makes it different from UWorld, Archer, SimpleNursing and Osmosis is not price alone. Archer's NCLEX plans run $129–$399, Picmonic is $67 a year, and Nurse.org sells TEAS prep at $19.99 a month, so cheap already exists. Level Up RN already sells med-surg flashcards and review games, so pretty cards alone are not an empty market either. The edge is five things together: free to start, already made (nothing to build), honest about what was checked, designed for tired and distracted brains, and a signature 3D body that

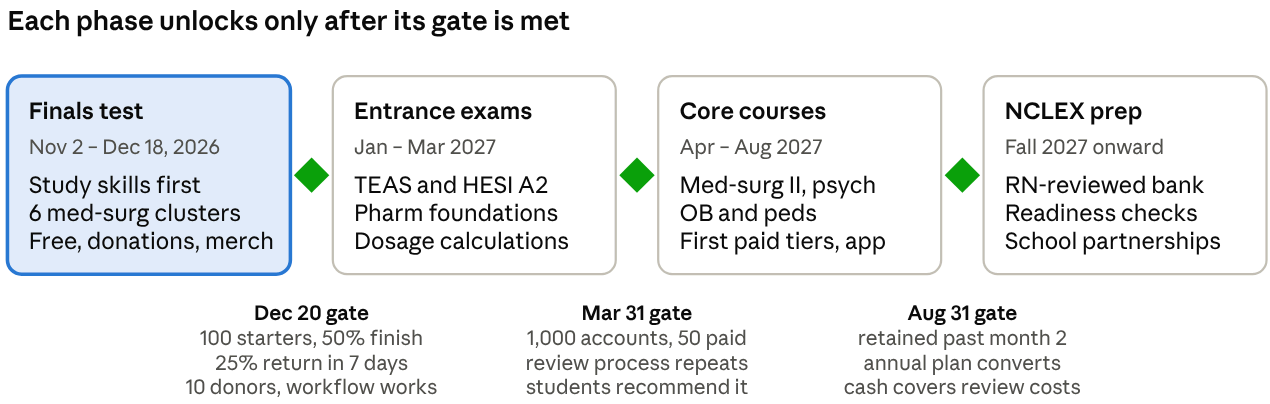
no competitor at this price owns. Elsevier's Osmosis AI, launched March 2026, already pairs AI with its own textbooks, so "AI plus textbooks" is not an edge you can hold; students accepting AI products (they do) is not the same as paying for one.

The first product is not clinical at all. "How to study for nursing school" is a short free course on how to learn, study and take nursing exams, and how the site supports each step. It carries no clinical-accuracy risk, can ship Monday, fits a faceless brand, and is the natural first thing to share with every nursing student regardless of semester.

Onboarding asks for five optional things: course level, exam date, topics on the exam, available study time per day, and a goal (catch up, practice, or both). No school passwords, no uploads of proprietary course packs, no patient data.

Phased roadmap

Four phases follow the student through school, and each later phase starts only after the gate before it is met. Everything else in the original idea (ER and ICU tracks, merch, brand equity deals, skills kits, sim labs) waits until there is traction.



roadmap · 4 phases, 3 gates

The green diamonds are decision points, not deadlines for features; if a gate is missed, the next phase does not start and the plan is revised or stopped.

Monday, November 2 launch package: the brand (working name, logo, handles, landing page with the "still building, more coming" notice), lesson 1 of the study-skills course live and free, three shorts cut from it, a tip page and the print-on-demand store with two or three items, the email list, and a working guest study flow on one page. No clinical content on Monday. The first med-surg lesson (fluid balance) goes live the first day it clears the full review workflow, targeted for November 9.

The 30-day plan (October 31 to November 28), in weekly blocks:

1. **Oct 31 – Nov 2**: name check and handles, landing page, study-skills lesson 1, first three shorts, tip page, store, analytics, bookkeeping file. Agents set up: content pipeline, build pipeline, review pipeline.
2. **Nov 3 – 9**: study-skills lessons 2–4, fluid-balance pack through review, guest study modes (learn, cards, questions), account saving, the 3D body embedded on one lesson. Three shorts a day from here on.
3. **Nov 10 – 16**: electrolytes and oxygenation packs, matching and fill-in modes, adaptive practice beta, break reminders, the developer page for Christian.
4. **Nov 17 – 23**: perfusion and glucose packs, organizers, reserved learning checks, the Arizona jurisdiction letters sent, ad slots designed (empty), first sponsor outreach list.
5. **Nov 24 – 28**: clinical-reasoning cases, corrections pass on everything, backups, installable web app polished, Google Play listing if donations have covered the $25, December runway planned.

Selling or promising access through a date is a commitment to keep the site running and corrected through that date.

The product: the atomic-sentence study engine

Christian's own study method becomes the product. Every lesson is a sequence of 15–25 short, standalone sentences that build from basic physiology to the bedside action, and every study mode on the site is a different way to work the same sentences. One concept, one ID, many modes; a concept that appears as a card, a blank and a question is still one concept, not three proofs of mastery.

The learning sequence: understand one idea, recall it, connect it to another idea, apply it in a case, revisit it later. The site recommends the next step but never locks the student into one mode or labels them a "visual learner."

Mode	What the student does	Safeguard
Learn	Reads one short explanation with a visual; "Why?", "Exceptions" and "Apply it" expand on tap	Simple wording must not turn a conditional judgment into an unsafe universal rule
Cards	Tries to recall before the answer is shown	Flipping a card is not evidence of mastery

Mode	What the student does	Safeguard
Matching	Connects terms to definitions or cues	Recognition is tracked separately from application
Fill in the blank	Recalls the missing value or relation	A clinically equivalent answer is not marked wrong
Organizer	Places cues, labs and actions into a labeled structure (condition, what changes, cues, priorities, actions, reassessment, escalation)	Tap-select and keyboard alternatives to dragging
Questions	Answers NCLEX-style items with rationales for every option	Distractors must be plausible and reviewed
Apply	Works a short original patient case	The case contains the information a defensible answer needs
Video	Watches a 3–5 minute lesson where the diagram builds as each sentence is spoken	Original visuals and script; nothing copied from Osmosis
Review	Revisits weak and overdue concepts on a schedule	No punitive streaks or forced daily use

Four additions from Christian's directions:

- **The study-skills course.** Five short free lessons: how nursing exams differ from other exams (application over recall), how to read a question for the cue that changes the answer, spaced review and why cramming fails, how to use cards, organizers and practice together, and exam-day routine. Each lesson ends by showing the matching site feature. It is the first product, the first set of shorts, and the thing to send any student who asks "where do I start."
- **The 3D body.** Z-Anatomy is an open-source 3D anatomy atlas (CC BY-SA 4.0, built on the BodyParts3D dataset, about 5,000 labeled structures) with a web viewer and source on GitHub. Embed it with credit, style the frame to the house look, and use it in every lesson and every short as the brand's signature visual: a recognizable object instead of a face. Adaptations of the model must be shared under the same license, so keep Cueory's own content separate from the model files. Do not recreate Kenhub, AnatomyLearning or GetBodySmart; their models and images are copyrighted.

- **Break and burnout care.** An optional Pomodoro-style timer (off by default, one tap to turn on, no clutter) suggests a break after 25–45 minutes with a short list of what to do (walk, water, stretch, a quiet minute), and a weekly check-in notices when someone is studying late every night and says so kindly. No punitive streaks, no guilt copy. Break suggestions are wellbeing tips, not medical advice.
- **The developer page.** A simple admin area for Christian: design tokens (colors, fonts, spacing) editable in one place, content in plain files he can edit in a browser (a git-based CMS such as Decap, free), a publish button, and a status board showing what the agents did overnight. Nothing technical required to change the look or the words.

Adaptive practice, honestly labeled. Month one uses simple rules: a wrong or hinted answer shows the rationale and then a different question on the same concept; several unassisted successes unlock a more applied item or a new topic; a concept comes back around days 1, 3, 7 and 14, adjusted toward the exam date; too little evidence shows "not enough practice yet," not a mastery percentage. Students can override the recommendation, and slow answers are not treated as low ability. Call it "Adaptive practice — beta."

The Argon proposal's Item Response Theory engine is the right long-term direction but the wrong first milestone. NCSBN's own CAT depends on item difficulties established by pretesting thousands of candidates; an AI-assigned difficulty label is not that. Never claim "same as NCLEX," a validated readiness score, or a probability of passing. Revisit calibrated testing when there are enough real responses per item and a psychometrician to design it.

The 40 reserved items (two 20-question learning checks) stay out of ordinary practice so they can measure something. They are learning checks, not predictors, and score changes are not marketed as proof the product works.

Content supply and rights

Build from Open RN, use the 1GB of textbooks as a reference shelf, and treat the Osmosis membership as competitor research. Both proposals reach the same conclusion, and it matches what was verified earlier: facts are free, expression is not, and attribution is not permission. This is general information, not legal advice; an hour with an intellectual property attorney before the paid product launches is cheap insurance.

Source	Use	Why
Open RN / WisTech Open (Nursing Fundamentals, Pharmacology 2e, Health Alterations, Skills, Mental Health, Nursing Assistant)	Adapt and sell, with credit, a license link and a note of changes	CC BY 4.0 permits commercial adaptation. Check each embedded image, video and activity for exceptions
OpenStax nursing series (8 books, 2024) and Anatomy & Physiology 2e	Confirm the license on each book page before using it commercially	The book prefaces say CC BY; OpenStax's help center says its textbooks are CC BY-NC-SA. Save a copy of each book's license page
Current clinical guidelines, FDA drug labels (DailyMed), the 2026 NCLEX-RN test plan	Verify time-sensitive claims; use the test plan as the topic checklist	Open textbooks age; drug facts, thresholds and isolation guidance need a current check
The 1GB of purchased textbooks and NCLEX books	Reference only: fact-check drafts, find gaps	Owning a book does not transfer its copyright. Do not feed chapters into AI, and do not reuse their questions, images, tables or mnemonics
Osmosis membership	Competitor research only	Elsevier has owned Osmosis since 2021; its terms limit use to personal, noncommercial, and its site reserves rights over text and data mining and AI training
Instructor slides, quiz banks, recalled exam questions	Never	Not yours to sell, and recalled licensure questions are a separate problem

"From Elsevier" and "inspired by" labels do not help. Naming the source on a copied or closely paraphrased question is a written admission, and putting a competitor's trademark on your product page invites a separate claim. Changing a name, an age and the answer order does not make a question original either.

The clean-room pipeline (Argon's best idea, kept): extract the bare clinical rule as a single pronoun-free sentence from your reference shelf ("IV calcium gluconate stabilizes the cardiac membrane first in severe hyperkalemia with peaked T waves; insulin and dextrose then shift potassium into cells"), then have AI write a brand-new scenario, four answer choices and rationales grounded in the matching Open RN chapter. The patient story, the wording and the distractors are new; only the unprotected fact carries over. Every item then goes through human clinical review before it is public.

Content supply lanes for month one: commercially reusable OER (audit, adapt, review); original material from a nurse educator under a written agreement; AI drafts from approved fact briefs; licensed proprietary collections investigated but kept off the launch path.

Credit template, filled with the exact source: "Adapted from [chapter] in Nursing Health Alterations, Open RN / WisTech Open, licensed under CC BY 4.0. Changes: revised presentation and added practice. The original creators do not endorse this platform." Keep a rights record for every reused element: source, location, license, permitted uses, required attribution, restrictions.

Students and AI. Christian is right that students will not object to AI-made materials; a 2025 survey found 92% of surveyed nursing students already use generative AI for coursework. The limit is not student taste, it is accuracy and trust. In one study GPT-4 answered 88.7% of NCLEX-RN practice questions correctly, which still leaves about one in nine wrong; a wrong drug fact on a flashcard is a real problem. The sale is "done for you and checked by a nurse," not "made with AI."

Content review policy: tiered, honest, no RN on payroll

Every item passes four checks (rights, accuracy, assessment quality, honest representation) through a workflow that is mostly automated, with Christian as the human editor and a qualified professional only for higher-risk content. This follows the GPT 6 Pro content-review policy of October 30, which found no blanket law requiring an RN to sign every question on an independent study site, while noting that Arizona regulates nursing education and licensure-prep programs. That classification is resolved by asking, not assuming: the policy's draft letters to the Arizona Board of Nursing and Arizona Private Postsecondary Education go out in week four.

The workflow, run by agents with a human gate at the end:

1. **Evidence first.** Pick a learning objective, assemble the permitted sources (Open RN chapter, current guideline or drug label where relevant), and build a small approved fact set. Never start by paraphrasing a proprietary question.
2. **Draft.** One model writes the lesson, cards, blanks, organizer and original questions from that fact set, keeping source content and Cuerie's new scenarios distinct.
3. **Independent solve.** A second model (different provider) gets the question and the references without the answer key, solves it, and says whether another option could also be right.
4. **Evidence audit.** A third model checks every important claim against the actual source text: supported, contradicted, or not established. "Not established" means hold.
5. **Mechanical tests.** Plain code checks units, arithmetic, answer alignment, duplicate options, missing rationales, broken links and rendering.
6. **Human editorial pass.** Christian checks each released item against its evidence record and the rendered page, and stops when a question exceeds his competence.
7. **Risk-based escalation.** Priority actions, emergency decisions, contraindications, dosing and infusion rates, delegation and procedure steps go to a qualified professional before release, or are deferred. A nursing educator for nursing judgment, a pharmacist for drug details.
8. **Release and correct.** Publish the reviewed version with its sources, review label, version and report button. A material change restarts the checks. Model agreement never overrides a missing source or an unresolved safety concern.

Risk level	Examples	Handling in November
Lower	Terminology, anatomy labels, documented definitions, study skills	Full automated workflow plus Christian's pass; ships
Intermediate	Simplified explanations, concept comparisons, original distractors	Same, with detailed source mapping; uncertainty escalated or cut
Higher	First actions, emergency algorithms, dosing, contraindications, delegation, procedures	Deferred until a qualified reviewer is available; not published in November

Christian's question pipeline, mapped onto the workflow. Questions are generated in batches of 250 (stem, four options, answer, rationale, concept ID, sources) from approved fact sets built on Open RN. Five separate ChatGPT Pro chats each review the full batch

blind, without the answer key, and an item survives only if all five agree with the key and with each other. Survivors go to Codex for the mechanical and consistency review (answer alignment, duplicates, units, rationale completeness, source links resolve). What passes goes to a final Claude pass (the strongest model available, high effort) that reads each item against its cited source text. Only then does the item enter the system, still carrying its risk level and the honest public label. Scripts, cards and organizers follow the same route with the same record. The 1GB of textbooks sits beside this pipeline as the verification shelf the checkers consult, not as the text the generator is fed; the moment they become source material the rights risk returns.

The developer table. Every item appears on the developer page in one sortable, editable table: ID, concept, risk level, stem, answer, rationale, sources, and one column per reviewer (GPT chats 1–5, Codex, Claude final, Christian) showing pass, fail or pending, plus version and public label. Inline edits create a new version and reset the downstream checks. The same table exists for scripts, cards and organizers.

Evidence is "enough to support the claim," not a quota: a definition needs one reliable reference with an exact location; a medication fact needs current labeling; a contested claim needs a balanced note. Three papers from one trial are one source. AI-invented citations are never used; every reference is resolved and inspected.

Public labels describe actual work: **Source-checked**, **AI-assisted**, **Clinician-reviewed** (named, scoped), and **Not independently clinician-reviewed**. Never "RN-reviewed" because the upstream textbook had RN authors, never "100% accurate," never a pass probability.

Professional review without hiring: a nurse educator or pharmacist reviewing a handful of higher-risk items for credit on the site and a share of later revenue is a contractor arrangement, not employment, and fits the rule of no nursing hires before $10,000 in revenue. Until someone like that is found, higher-risk content stays out.

Brand and name

Cueory is the working name (pronounced KYOO-uh-ree: cue plus theory), with Cluerie as the backup. The GPT 6 Pro branding review could not confirm that cueory.com or the @cueory handles are available, so week one starts with a registrar check, the handle claims and a comprehensive trademark search for similar-sounding marks in the education, software and apparel classes. It avoids the problem Cuerie had: "Curie Learning" holds a registered education-class trademark, and Cuerie sounds identical to it. Merch and any filing wait for the search.

Why the name works: nurses recognize cues, manage cues and analyze cues all day (the NCLEX clinical-judgment model literally starts with "recognize cues"), and the name says the site turns cues into understanding. Tagline: "Nursing study, made clear." Sign-off line for every short, to test against two others in week one: "Know your cues." Use it constantly for the first month so the meaning sticks. One known weakness: people may hear "query," so the spelling appears on screen at the end of every video.

Murse is retired as a brand: the domain is held by an investor, the word divides the profession, it also means a man's handbag, and there is prior apparel use ("Murse World," acquired by AvaCare in 2018, and a 2023 "DR. MURSE" filing). The motto stays: by nursing students, for nursing students. The founder line is "Founded by a nursing student. Built for nursing students," which is true without a face or a title.

Design direction: minimal, urban, classy. Generous whitespace, one strong typeface pairing, large legible study text, muted palette (warm ivory, charcoal, muted sage, one controlled accent), and restraint everywhere else. The references Christian named (recent federal sites with the same designer's hand) share exactly that: big type, few colors, nothing decorative. The marketing page can be expressive; the study screen is calm. Correctness and urgency never shown by color alone, generous touch targets, reduced-motion respected, visible keyboard focus. All of this lives in the design tokens on the developer page so it can change without a rebuild.

Signature visual: the 3D body plus the "cue highlight" motif, where one phrase lights up and a line connects it to the structure, finding or next step. That motif repeats across lessons, cards, organizers, shorts and merch.

Merch (print-on-demand, no inventory, no upfront cost): minimalist shirts, crewnecks and totes with the wordmark and short lines that nurses actually say to each other. Candidates to test: "Know your cues." "Recognize the cue." "Assess before you act." "Calm is a clinical skill." "Nurses notice." Keep the type small and the colors quiet so it reads as a brand people wear, not a slogan. Merch waits for the trademark check.

Faith, quietly: Catholic contributions to medicine are real and welcome here, offered as culture rather than preaching. Ways that fit the brand: a short "Why hospitals exist" lesson in the study-skills course (religious orders built the first hospitals and nursing schools), St. Camillus de Lellis and St. John of God as patrons of nurses noted on their feast days in social posts, a one-line blessing option on the break screen that a student can turn on, and, later, the single-parent scholarship. Nothing is forced on a student who did not ask for it; the brand stays open to everyone.

Pricing and revenue

Everything is free until a whole course is complete; until then the asks are a donation and a shirt. That matches how Christian wants to test it, costs nothing to run, and builds the audience that every later revenue line depends on.

Stage	What is free	How money comes in
Now through the first full course (target: January)	All lessons, practice, cards, organizers, the 3D body, the study-skills course	Tip page ("help build the starter fund"), print-on-demand merch, affiliate links on gear students already buy
After the first full course	Everything already published stays free	Supporter tier (about $3 a month or $25 a year: early access, print packs, name in the credits) and a Premium tier (about $7 a month or $49 a year: full courses, custom practice, learning checks, offline packs); hardship code for anyone who asks
Once traffic is real (roughly 10,000 monthly sessions)	Same	Native sponsors and designed ad slots, then brand partnerships
After outcomes exist	Same	Institutional licensing to nursing programs

Donations, done right. A tip page (Ko-fi, Buy Me a Coffee or a Stripe payment link) with a clear "this funds the next course" message. Donors get thanks and credits, never equity or a promised return; the moment contributors get a stake, it becomes a securities offering with SEC rules. Expect small numbers: a tip page converts well under 1% of visitors, so this is proof of goodwill more than funding.

Ads, in the house style. Christian is right that ads will be part of the income, and right that they have to look like the brand. Design the slots now (one quiet native placement on lesson index pages and the blog, a sponsor card in the weekly email, a sponsor mention at the end of shorts) and leave them empty until there is traffic worth selling. Nothing inside a study session, ever: an ad next to a quiz breaks the ADHD-friendly promise. Generic ad

networks pay little at this scale (a blogger with about 40,000 monthly sessions made at most $300–400 a month) and do not match the look; sell the slots directly to nursing-adjacent brands once the numbers justify a media kit.

Tiers later, not now. Card fees of 2.9% plus 30 cents make a $1 tier pointless (about 67 cents kept), which is why the lowest paid tier is $3 and annual plans are preferred. Any paid tier adds features; it never removes what was free. Discounts are standing, not occasional: a permanent student-hardship code, a founding-member rate locked for life for the first 500 subscribers, a semester bundle at the start of each term, and a referral month free for both students. A discount calendar is part of the Friday business report.

Marketing and video

Three shorts a day, faceless, from one template, each ending on "Stay cuerie" and a link to a free lesson. Volume is possible only because every short is cut from a master lesson by a pipeline, not made by hand.

The pipeline (agent-run, Christian approves): master lesson JSON (atomic sentences, visuals, sources) → three 20–45 second scripts per lesson (explain one concept, compare two confusable concepts, show the site doing it) → one consistent AI voice → a Remotion or CapCut template on the house palette where the 3D body or the cue-highlight diagram builds as each sentence is spoken → captions burned in → AI label applied → posted to TikTok, Instagram Reels and YouTube Shorts from a scheduler. Cost per short: a few cents of voice generation. Christian reviews a day's batch in ten minutes.

Three series, repeated forever: Make it make sense (one confusing concept), Spot the cue (a reviewed question, a pause, the answer), Already made (a real demo of cards, organizer or the 3D body). Add a weekly honest founder update in text and voice, no face, plus a patron-saint or history note where it fits.

AI label is fine, and required anyway. YouTube requires disclosure of realistic synthetic content and lists a synthetic voice narrating a video as an example; TikTok requires a label on AI content with realistic audio or video. The brand says so in the bio and on every post. The one thing to avoid is YouTube's inauthentic-content rule, which demonetizes whole channels that mass-produce near-identical videos: three a day is fine as long as each one teaches something distinct, which the three-series structure guarantees.

Monetizing the videos, honestly. Platform payouts are small (Shorts typically $0.01–$0.06 per 1,000 views; TikTok's Creator Rewards needs 10,000 followers, 100,000 views in 30 days and videos over a minute). Join both programs the day the thresholds are met, post a

one-minute version of each lesson for the TikTok and YouTube Partner thresholds, and treat the real money as later sponsor reads and affiliate links. Track every video to a completed practice session; views are context, not value.

Honesty rules: no fake students, invented testimonials, bought engagement or undisclosed affiliations. The FTC's reviews rule covers fake and AI-generated testimonials. Agents prepare and schedule; nothing posts without Christian's daily approval.

User data and tracking. Privacy-respecting analytics on every page and video link (which post brought the student, which lesson they finished, whether they came back, where they dropped off), plus the study attempts in the database. Collect only what is needed, publish a plain privacy page, and separate study reminders from marketing consent.

Month-one diagnostics (targets, not forecasts): 100 relevant learners who begin; at least 50% finish a first lesson plus five questions; at least 25% return within seven days; 15 voluntary classmate shares; 10 donations or merch orders; 1,000 followers across channels; zero unresolved serious content or security issues.

Build: stack, data and the AI team

Agents build and check the back end; Christian directs content and design from a simple developer page and approves what goes public. This is the division of labor he asked for, and it matches the crew he already runs in GitHub for ACTA: one provider builds, a different provider reviews, nothing merges to sensitive paths without his tap. The stack stays inside free tiers.

Layer	Choice
Interface	TypeScript and React, with reusable study components (card, blank, organizer, question)
Source control	Private GitHub repository; Claude Code as the primary build agent
Hosting	Cloudflare Pages or equivalent with confirmed commercial terms
Accounts and database	Supabase Auth and PostgreSQL with row-level security; its free plan lists a 500 MB database, enough for month one, not for 50,000 items plus logs

Layer	Choice
Question delivery	Server-side endpoints that pick reviewed items and check entitlements; never ship the whole bank in front-end JavaScript
Payments	A hosted checkout page with server-side entitlement updates
Monitoring	Basic error reporting and a small event funnel

Guest to account. For Monday, progress lives in the browser and the page says so. After account saving is tested, offer a free account to sync across devices, importing the guest session without resetting it; give every attempt an idempotency key so a retry does not double-count. Show the account prompt once per session, after a finished lesson or five questions, never mid-answer, with a visible "keep studying without an account" option. Use Google sign-in or a properly configured email provider; Supabase's default SMTP is not for public signups.

Minimum data model: Concept (ID, objective, topic, prerequisites, version), Item (concept IDs, stem, format, rationale, family ID, version, status), Rights record, Review record, Attempt (learner, item version, response, correctness, hints, mode, time, unique event ID), Review state (due date, evidence count, last recall), Exam plan, Entitlement. Keep answer keys and reserved checks separate from public listings. Collect only necessary personal data, separate reminders from marketing consent, provide deletion and export, and do not claim HIPAA or FERPA compliance without an actual assessment.

The AI crew, running on its own. Scheduled jobs (GitHub Actions on the existing setup, or the Oracle free-tier server already in use) run the content pipeline, the review workflow, the video pipeline and the build loop overnight. Each morning Christian gets one status board: what was drafted, what passed review, what is waiting for his approval, what failed. He approves in the browser; nothing he approves requires a terminal. Secrets live only in GitHub's secret settings or the server's locked file, never in chat.

The app path, inside the budget. The site ships as an installable web app from day one (students add it to the iPhone home screen; it works offline for cached lessons and sends reminders where the platform allows). Apple's developer account is $99 a year, which is the whole month's budget, so the iOS App Store listing waits until donations cover it; Google Play is a one-time $25 and comes first. The web app is built so the same code wraps into both stores (Capacitor), so "the app" is a packaging step, not a rebuild.

Finances tracked from day one. A free bookkeeping tool (Wave) connected to the payment accounts, every expense logged with a receipt, income split by source (donations, merch, affiliate, sponsors, subscriptions), and a monthly export. Form a single-member LLC when revenue justifies the Arizona filing fee, not before. The single-parent scholarship and any tax advantages that come with a nonprofit arm are real options after $10,000 in revenue and are noted in the long-term section; before that, a scholarship is just a gift.

Agent	Tool	Runs	Output
Source and rights analyst	Gemini with web research	Per lesson	Source brief and rights record
Lesson builder	ChatGPT	Nightly	Master lesson JSON: sentences, cards, blanks, matching, organizer, scripts
Question writer	Claude	Nightly	Original items from approved fact sets, with Bloom level
Independent solver	A different provider from the writer	Nightly	Solve-without-key report
Evidence auditor	Gemini with scite	Nightly	Supported / contradicted / not established, with locations
Platform engineer	Claude Code	Continuous	Features as small pull requests with tests
Code reviewer	Codex	On every pull request	Clean or not; bot merges non-sensitive paths
Video producer	Remotion or CapCut template plus TTS	Nightly	Three labeled shorts per lesson, scheduled
Marketing editor	Claude	Daily	Captions, carousels, weekly email, distribution log
Status reporter	Any	Each morning	The one-page board on the developer page

Agents prepare, check and schedule; Christian's approval is the only path to public. The human editorial pass in the review workflow cannot be delegated to an agent, and no clinical item ships without it.

Staging before production. Two copies of the site: a protected test site (staging) and the public site. Every change lands on staging first, where Christian can click through it; a "promote to live" button on the developer page moves it to production. Staging is behind a login (Cloudflare Access, free for a small team), is never indexed by search engines, uses a separate database with fake student data, and carries a visible "TEST" banner so nothing on it can be mistaken for the real site. Production deploys only from the main branch after Codex review and Christian's approval.

Security and privacy, designed in. Nothing here is exotic; it is the standard set, applied without exceptions:

- Accounts through Supabase Auth with Google sign-in and email magic links; no passwords stored by Cueory; optional two-factor for the developer login, required for Christian.
- Row-level security so a student can read only their own progress; answer keys and reserved checks served only from the server, never shipped to the browser.
- Secrets only in GitHub secret settings or the server's locked file; automatic secret scanning on every commit; keys rotated on any suspicion.
- Rate limiting and bot protection on sign-up, login, the report button and the tip page (Cloudflare, free tier); input validation on every form; content security policy headers; HTTPS only.
- Dependency and vulnerability scanning on every pull request (GitHub Dependabot and CodeQL, free for the repo); automated tests must pass before anything merges.
- Minimal data: email, display name, study progress, the onboarding answers, nothing else; a plain privacy page; delete-my-account and export-my-data buttons that work; no selling data, ever; analytics that do not track across sites.
- Daily encrypted database backups with a tested restore; an incident plan of three lines: take the affected part offline, tell users plainly within 72 hours, fix and post what changed.
- Christian's own devices: a password manager, two-factor on GitHub, Supabase, Cloudflare and the registrar, and a habit of never pasting keys into chats.

Agent teams and token cost. Five small teams, each a set of role prompts and scheduled jobs rather than always-on agents, so tokens are spent only when there is work:

Team	Jobs	Cadence	Models (cost tier)
Research	Source briefs, rights records, evidence audits, competitor watch	Per lesson	Gemini (free plan), scite
Content	Lessons, questions, scripts, the five-chat review, cards, organizers	Nightly batches	ChatGPT Pro for drafting and the five reviews; Claude for the final pass
IT and development	Features, tests, security scans, staging deploys, backups, status board	Continuous on pull requests	Claude Code (Opus) builds; Codex reviews; Sonnet-class model for routine chores
Marketing	Three shorts a day, captions, weekly email, polls, distribution log, sponsor list	Daily batch	Sonnet-class model plus the video template; Claude for the weekly plan
Business and finance	Bookkeeping categorization, monthly numbers, pricing tests, the gate report, jurisdiction letters, discount calendar	Weekly	Claude (high effort) once a week; a spreadsheet the rest of the time

The expensive models (Opus-class for building, the strongest Claude for the final content pass and security design) are used where errors cost the most; everything repetitive runs on the cheaper tiers. Expect the paid plans already in place to cover this month; token spend is reviewed in the Friday business report.

Budget, hours and month-one targets

Cash spend in the next 30 days is capped at $100, and the plan fits under it; everything else is time and the AI plans already paid for. Anything beyond $100 waits for donations or sales to cover it.

Item	Cost	When
Domain (cuerie.app or similar)	$15–30 for the year	Week one
Hosting, database, checkout, analytics, bookkeeping, CMS, scheduler	$0 on free tiers at this scale	Now
AI voice generation for about 90 shorts	$0–15 (free tiers or a self-hosted open voice model on the existing server)	Ongoing
Print-on-demand store	$0 upfront; the printer takes its cut per order	Week one
Google Play developer account	$25 one-time	When donations cover it
Apple developer account	$99 a year	After donations cover it; not this month
Trademark search (DIY on USPTO)	$0; a filing is $350 per class later	Week one for the search
Professional review of higher-risk items	$0 this month, because higher-risk items are deferred	After a reviewer is found
Reserve	Whatever is left of the $100	Untouched unless something breaks

The AI subscriptions already in place (Claude Max, ChatGPT Pro, Google AI Pro, about $300 for the sprint) are the real production budget and are sunk; they step down December 1 unless results justify them, so this month has to produce something visible by then.

Founder hours at about 12 a day: roughly a third on content direction and the daily approval of lessons and shorts, a third on design and the developer page, and a third on distribution, replies and watching the numbers. Technical setup is the agents' job; Christian's technical work is limited to pasting commands the agents give him, one block at a time, and tapping approvals.

Month-one production targets (maximums, scaled down to what passes review): the 5-lesson study-skills course, 6 med-surg topic clusters, 12 reviewed micro-lessons, 180 unique reviewed lower- and intermediate-risk items (140 in practice, 40 reserved), about

120 card prompts, 12 organizers, the 3D body on every lesson, about 90 shorts, and six study modes live.

One honest note on money: a free product with a tip jar, launched in November, produces tens of dollars in its first month at best. It is built alongside an income, not instead of one, and the AI plans cost more per month than the site will earn this year unless the December gate is cleared decisively.

Long-term vision, with proof gates

The Argon proposal's five-year arc (skills kits, simulation studios, institutional licensing, an accredited hybrid nursing program, then medical education) is a coherent direction, and none of it belongs in this year's scope. Each stage is unlocked by proof from the one before it.

Stage	What it is	Proof needed first
Finals study platform (now)	Reviewed med-surg lessons and practice	Real use, safe content, repeat visits, purchases
Full nursing curriculum (2027)	Fundamentals through NCLEX	A repeatable review process and sustainable production cost
Human skills videos (2027–28)	Practicing RNs filming check-off skills on revenue share	Qualified educators, consent, quality control, cash to pay them
At-home skills kits	IV, Foley, med-draw and suture practice kits with QR-linked video rubrics	Demand signal from students, supplier terms, liability insurance
Institutional licensing	The platform sold to community colleges and BSN programs as a cheaper alternative to ATI and HESI packages	Demonstrated outcomes and an institution's procurement requirements
Simulation studios	Bookable practice bays with mannequins and RN coaches, starting in the Phoenix metro	Insurance, qualified staff, equipment, lease economics, and clarity that practice time is not clinical credit

Stage	What it is	Proof needed first
Accredited hybrid program	A degree-granting nursing program with hospital partners subsidizing tuition	Separate regulatory, faculty, clinical-placement and accreditation planning; Arizona's distance-learning rule requires supervised on-ground clinical and lab hours
Medical education	The same engine for MCAT, USMLE and COMLEX	Medical leadership, its own standards and approvals

The most plausible near-term extensions are the skills videos and the kits: both reuse the audience and the content pipeline, and both can be tested with a presale before any inventory is bought. Licensing to schools is the most valuable, and it depends on outcome data the finals test is designed to start collecting. An accredited school is a different business with different regulators; partnering with approved schools to improve learning and skills preparation is the realistic version of that ambition.

Brand partnerships (Figs, Alo, Lululemon and the like) come after the audience exists: a media kit with real numbers, a few organic posts where the products appear naturally, then a pitch. Scrubs brands sponsor nursing creators routinely; they want reach and a clean aesthetic, both of which this plan builds.

After $10,000 in revenue: hire or contract an RN educator, form the LLC if not done, and look at a nonprofit arm for the single-parent scholarship. Donations to a 501(c)(3) are deductible for donors and the scholarship itself becomes a program expense; a CPA should confirm the structure before anything is promised publicly. Until then, no nursing hires and no tax claims.

Risks and what would kill this

Risk	How likely	What it does	Mitigation
A wrong clinical fact goes public	Medium	Destroys trust with the exact audience that spreads word of mouth	Tiered workflow, higher-risk content deferred, honest labels, report button, pull-and-fix rule

Risk	How likely	What it does	Mitigation
The name fails clearance	Medium	Rebrand after merch and handles exist	Search in week one; merch and filing wait; two backups ready
A takedown or legal letter from a publisher	Low if the content rules are followed; high if the textbooks or Osmosis become source material	Content removed at the worst moment	Open RN base, clean-room pipeline, reference-only shelf, source register
Arizona treats the site as a regulated program	Low for a self-study publisher; unresolved	Would require approval or a change of scope	Jurisdiction letters in week four; no credit, certification or CE claims
Nobody comes	Medium	90 shorts into silence	Three series, topic polls, the 3D body as a hook; the December gate catches it early
They come but do not return	Medium	Finals urgency does not become a habit	Review reminders tied to the exam date, break care, an account worth having
Agents produce volume without quality	Medium	Templated videos, repetitive items, a demonetized channel	Three distinct series, review workflow, Christian's daily ten-minute approval
Founder burnout	High	12-hour days with tens of dollars back	Scope tied to what passes review; the gates give explicit permission to stop; the site's own break rules apply to its founder

Risk	How likely	What it does	Mitigation
AI plans step down December 1	Certain unless justified	Production pipeline slows	Front-load content in November; keep the master-lesson format so slower tools still work
Platforms change the rules	Medium	Reach or payouts lost	Own the email list and the site; video is marketing, not the business

Two things would end the venture rather than wound it: publishing higher-risk clinical content without qualified review, and building the content base from Elsevier's material. Neither is a risk to manage; both are lines not to cross.

Verdict: is this worth taking on?

Yes, as a 30-day all-in test followed by a December 20 decision, and under $100 it is one of the cheapest tests of a real business idea available. Not as the five-year empire, and not as income this quarter.

Why yes:

- The problem is real and the market is large. About 193,000 first-time U.S.-educated candidates took the NCLEX-RN in 2025, and nearly all of them paid for study help on the way.
- It fits Christian's strengths. The product is the study method he already uses, made for people who do not have time to make their own materials, and he has already built exactly this kind of visually polished study material for his own exams.
- The cost of finding out is low. Under $1,000 in cash, no inventory, free hosting at this scale, and open textbooks that can legally be adapted and sold.
- Both independent proposals converge on the same plan. When two different analyses, with different risk appetites, agree on the pivot, the method, the content base and the pricing, the core idea is sound.

Why only as a test:

- Higher-risk clinical content cannot ship until a qualified reviewer is found, so the November catalog is concepts and assessment, not the full course students eventually want.
- The winning edge is trust, design, the 3D body and being done-for-you, not price. Cheap prep already exists, and the big platforms already have AI. That edge takes months of consistent, visible work to build, and it cannot be bought.
- It will not replace an income this year. A free product with a $5 pass starting in November produces pocket money through the spring at best; it is a side project that earns the right to become a business.
- The ambitious parts of the idea (sim labs, an accredited school, celebrity brand deals) are not reasons to start; they are things a successful version might one day do.

Conditions to start:

- [ ] The name clearance search is run in week one; merch and any filing wait for it.
- [ ] Content is built only from Open RN, verified-license OpenStax titles and original writing; the textbook shelf and Osmosis are reference and research only.
- [ ] Higher-risk clinical content (first actions, emergencies, dosing, contraindications, procedures) stays unpublished until a qualified reviewer exists.
- [ ] Every public item carries its sources and an honest review label; every video carries the AI label.
- [ ] Cash spend stays under $100 unless donations or sales cover the extra.
- [ ] The December 20 decision uses the gate numbers: 100 starters, 50% finishing a lesson, 25% returning within a week, 10 donations or orders, and a review workflow that actually ran.

If the December gate is met, the TEAS and HESI release in January is already planned and the plan continues. If it is missed by a little, change the angle (topic, format or channel) and test one more cycle. If it is missed by a lot, stop, having spent under $1,000 and learned something true about the market.

Sources

Inputs: the Argon 4 strategy proposal, the GPT 6 Pro strategy v2, the GPT 6 Pro branding and naming review (Cueory, LessonForm, Cluerie), the GPT 6 Pro content-review and legal-boundaries policy, and Christian's handoff notes and 29 directions, all dated

October 30, 2026. Pages below were opened during this and earlier research; Arizona statutes, Elsevier terms and the three AI-question studies are cited from the policy document's source list.

- cuerie.com host and WHOIS record
- CURIE LEARNING registered trademark, class 041
- CURIEUX trademark filing, patient-education software
- Z-Anatomy open-source 3D atlas, CC BY-SA 4.0
- Z-Anatomy web viewer and source
- AvaCare Medical acquires Murse World (2018)
- Cox, Hunt and Hill 2023, AI vs educator NCLEX-RN questions
- Law et al. 2025, AI vs human exam questions
- Al-Najafi et al. 2026, AI-generated questions RCT
- A.R.S. 32-1601, nursing definitions
- A.R.S. 32-1666, unlicensed practice and unapproved programs
- A.R.S. 32-3021, private postsecondary exemptions
- NCSBN 2025 NCLEX statistics, via GoodNurse
- AACN 2025–26 enrollment data, men in nursing
- Archer Review NCLEX pricing (TestPrepPal)
- Picmonic vs SimpleNursing pricing
- Nurse.org TEAS prep pricing
- Elsevier launches Osmosis AI, March 2026
- Elsevier acquires Osmosis, 2021
- Osmosis site rights notice
- Elsevier Osmosis terms and conditions
- Elsevier HESI
- Open RN license terms (Nursing Advanced Skills preface)
- Open RN project overview (CVTC)
- OpenStax licensing help article
- OpenStax Clinical Nursing Skills preface (CC BY statement)
- What copyright does not protect (Copyright Alliance)
- Nursing students' use of generative AI, 2025
- ChatGPT performance on NCLEX-RN questions (JMIR Medical Education, 2024)

- YouTube: disclosing altered or synthetic content
- YouTube inauthentic content policy, July 2025
- TikTok AI-generated content labels
- TikTok Creator Rewards eligibility
- YouTube Shorts RPM ranges
- Mediavine requirements 2026
- Stripe fees (NerdWallet)
- Regulation Crowdfunding overview
- murse.com host and WHOIS record
- DR. MURSE trademark filing
- What is a male nurse called (Nurse Money Talk)
- Maricopa Community Colleges important dates
- NCSBN computerized adaptive testing
- NCSBN 2026 NCLEX-RN test plan
- Arizona Administrative Code, Title 4, Chapter 19 (Board of Nursing)
- FTC consumer reviews and testimonials rule